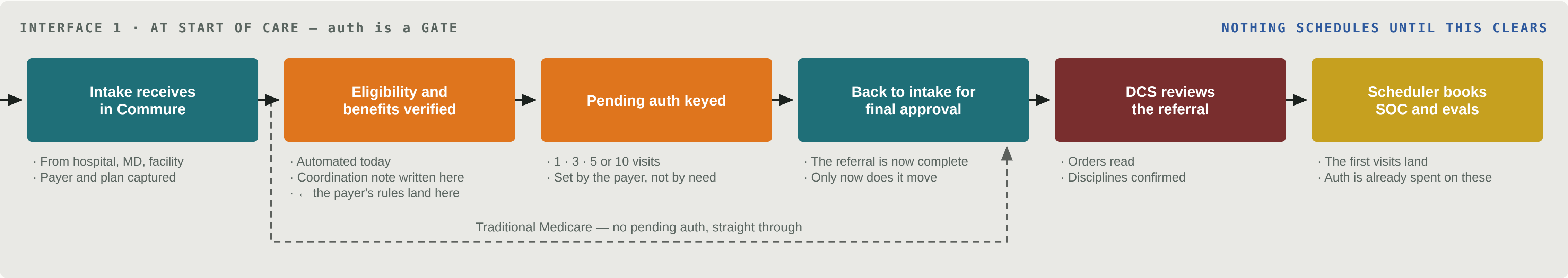


Authorization

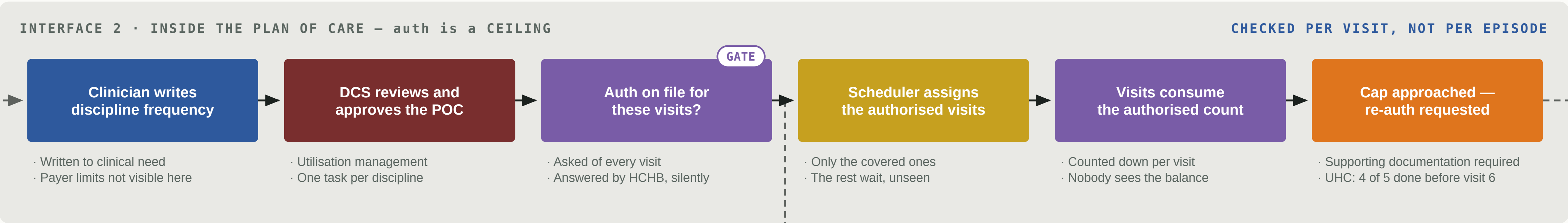
Two interfaces only — the gate before the first visit, and the ceiling inside the episode

TRIGGER

Referral arrives hospital · MD · facility



“We know we have the referral, but it is just not in my workflow to schedule yet — it is stuck in auth.”



NO AUTH — the visit sits pending

It is not on the clinician's calendar, it does not count toward productivity, and it lives in the scheduler's head or on a sticky note.

Every add-on order, recertification and resumption of care re-enters this loop — and each one is a new auth question

WHAT THE PAYER HAS ALREADY DECIDED — before anyone writes a plan of care

THE CAP IS SET UPSTREAM

- UHC — 5 nursing visits; 4 of 5 completed plus documentation before visit 6
- Indiana Medicaid — 8 visits shared across PT / OT / ST
- Indiana Medicaid — 30 days from the discharge date, not the admit date
- Ohio Medicaid — caps similarly

TWO CEILINGS, NOT ONE

- Auth is permission — how many visits the payer will allow
- PDGM is payment — the economic band for the period
- LUPA is the floor; utilisation management is the ceiling
- A visit can be authorised and still be uneconomic

The data already exists. The auth team writes the payer's rules into a coordination note at verification — days before anyone writes the plan of care.

Surfacing those rules at plan-of-care creation is the highest-value, lowest-complexity win in this flow — and a patient-care win, because abrupt discharges happen when nobody planned for the real visit budget.

BOUNDARIES

Auth is drawn only where it meets scheduling

Traditional Medicare has no pending auth

Auth governs permission · PDGM governs payment

WHERE IT BREAKS

Pending-auth visits are invisible not on a calendar, not counted

~50 pending-auth workflows a day so schedulers bulk-clear without reading

Plans of care ignore payer limits then the team is surprised

Abrupt discharge when the visit budget quietly runs out